Depressive and Bipolar Disorders ✕

Today's goal • 3/3 items

Module 1
Learn With PsycLearn Essentials

Module 2
Opening to Depressive and Bipolar Disorders

Module 3
Depressive and Bipolar Disorders

Module 4
Biological Contributing Factors

Module 5
Psychological Contributing Factors

Module 6
Treatments for Depressive and Bipolar Disorders

Lesson 9: Biological Treatments
- ✓ Overview: Biological Treatments
  Reading • 5 min
- ✓ Medications and Neurotransmitters
  Reading • 10 min
- ✓ Brain Stimulation Therapies
  Reading • 10 min
- ✓ Check Your Understanding: Biological Treatments for Depression
  Practice Assignment • Grade: 100%

Lesson 10: Psychological Treatments
- ○ Psychotherapy
  Reading • 5 min
- ○ Cognitive Behavioral Therapy
  Reading • 5 min
- ○ Cognitive-Behavioral Activity Review
  Reading • 5 min
- ○ Practical Scenario
  Real-world application • 5 min
- ○ Third Wave Cognitive-Behavioral Therapy
  Reading • 5 min
- ○ Psychological Treatments in the Digital Age
  Reading • 3 min
- ○ Integrating Treatments for Mood Disorders
  Reading • 10 min
- ○ Check Your Understanding: Psychological Treatments for Depression
  Practice Assignment • 11 min

Key Takeaways
- ○ Key Takeaways: Treatments for Depressive and Bipolar Disorders
  Reading • 10 min

Module 6 Quiz
- ○ Module Quiz: Treatments for Depressive and Bipolar Disorders
  Graded Assignment • 10 min

Module 7
Suicide

Module 8
Conclusion

Module 9
Cumulative Quiz

Module 10
Course Resources

Module 11
PsycLearn Essentials APA Student Resources

Psychotherapy

0:05 / 6:39 1x

Psychotherapy

Psychological treatments are usually referred to as psychotherapy. **Psychotherapy** is any psychological service provided by a trained professional that primarily uses forms of communication and interaction to assess, diagnose, and treat dysfunctional emotional reactions, ways of thinking, and behavior patterns. Psychotherapy may be provided to individuals, couples, families, or members of a group. These "talk therapies" typically involve meetings between the therapist and client face to face in a room, or in a video chat room, to discuss the client's thoughts, feelings, and behaviors in an effort to identify and treat the sources of the client's depression. A clinician's particular orientation to psychotherapy directs the strategy that is adopted to address the client's distress. For depression, cognitive behavioral therapies are the most common approach.

Psychoanalytic Therapy

Psychoanalysis, developed by Sigmund Freud, stresses the importance of unconscious feelings and motives from childhood that drive behavior and produce psychological disorders. Psychoanalysis has fallen out of favor in recent decades. Two major reasons for the decline in popularity are time and money. Clients may have two to five sessions each week for many years, resulting in a cost that is far too expensive for most people to afford. Another major problem for psychoanalysis is a lack of empirical evidence supporting its efficacy. Though there are case studies demonstrating successful outcomes, no research has compared the outcomes of participants receiving treatment with those not receiving treatment. Without this comparison it is impossible to conclude that psychoanalysis is effective. Finally, classic psychoanalysis is outdated in that it incorporates elements of sexism, heterosexism/homophobia, and racism (Balsam, 2015; Stoute, 2022).

Behavioral Therapy

In the 1940s, a group of psychologists abandoned the psychoanalytic approach advocated by Freud and began to focus, instead, on observable behavior. These behavioral psychologists believed that behavior is learned through the associations that develop from individuals' interactions with their environment. Behavioral therapists framed disorders such as depression and anxiety as maladaptive behaviors (i.e., behavior that is detrimental or counterproductive), and applied learning principles as the central element of their clinical practice.

Behavioral psychologists focus on the learning of behavior through conditioning, which is the formation of associations that influence behavior. Behaviorists have identified three types of learning: classical conditioning, operant conditioning, and observational learning. In classical conditioning, an association between a neutral stimulus and a response-producing stimulus can influence automatic, involuntary behavior. In operant conditioning, the association between a behavior and a consequence can influence the likelihood of producing a voluntary behavior given the same conditions in the environment. Finally, observational learning is learning from watching others' behaviors and allowing it to influence our behavior.

Cognitive Therapy

By the 1960s, though, many psychologists had come to believe that focusing exclusively on behavior was ignoring another significant influence on behavior: thoughts. These cognitive psychologists were beginning to examine how people process information and how thoughts and feelings can both help and hurt mental health. They proposed that information processing is a key component of human understanding. Cognitive therapists began to treat psychological disorders by focusing on faulty thinking and distorted beliefs. In general, cognitive therapy uses a set of techniques to change maladaptive beliefs and thought processes, including setting realistic goals, cognitive restructuring, and guided discovery.

- For **goal setting,** the therapist helps the client develop realistic goals. A client with social phobia who has a fear of going out in public might set a series of goals regarding their social behavior. They might decide to (a) go to the grocery store once per week and make conversation and eye contact with the cashier, (b) spend one hour in a coffee shop and interact with at least one person there, and (c) go out to one restaurant or bar after dark and interact with at least two people there. Once goals have been set, interventions target the biased beliefs and cognitive distortions underlying disorders. For example, with social phobia, biased beliefs might include "No one likes me" and "Meeting new people is a waste of time." Though the specific beliefs will vary from one disorder to another, the goal is the same: to get clients to examine the beliefs, assumptions, and automatic thoughts that are causing them distress.
- Once the therapist and client set goals and identify biased beliefs, the therapist will work with the client to engage in **cognitive restructuring.** The therapist will typically ask clients to provide evidence to support the accuracy of their biased beliefs and generate explanations for behavior that are not maladaptive. For example, a person with social phobia may have cognitive distortions when a store clerk does not smile at them during check-out. Their automatic beliefs might include: "The clerk didn't like me" and "I must look terrible." The therapist will work with the client to generate alternative explanations for the behavior of the unfriendly clerk. For example, "The clerk was having a bad day" and "The clerk isn't friendly to anyone" are two unbiased alternative explanations. The goal of restructuring beliefs is to break the link between a particular situation and the accompanying negative thoughts.
- **Guided discovery** involves the therapist asking clients questions that help them view events in a new way or to consider information outside of their awareness. Clients in guided discovery often have responses such as "I hadn't thought of it like that before" or "I didn't think of that." Guided discovery teaches clients to examine the accuracy or rationality of their beliefs and to develop more adaptive beliefs.

Though the behavioral and cognitive approaches are distinct perspectives, contemporary clinicians most often use them in combination, both to explain human behavior and to treat individuals with psychological disorders.

Mark as completed

Like Dislike Report an issue

Go to next item →